This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Aciclovir or Valaciclovir tablets for the treatment of Genital Herpes

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Genital Herpes Management

Overview

- **Genital herpes (HSV-1 and HSV-2):** A lifelong infection with unpredictable recurrences. Most people have mild to moderate symptoms, but complications can occur.
- **Transmission:** Sexual contact, highest risk during prodrome and active lesions. Can be transmitted even without symptoms (asymptomatic shedding).

Assessment

- **Clinical presentation:** Painful vesicles progressing to shallow ulcers, fever, malaise, dysuria, inguinal lymphadenopathy.
- **Viral swab:** PCR or culture for confirmation (vesicular fluid or base of ulcer).
- **Sexual history and risk factors:** Number of partners, contraception use, STI screening, frequency of recurrences (determine if suppressive therapy indicated).
- **Frequency of recurrences:** 6 or more per year = consider suppressive therapy.

Management

- **First episode:** Antiviral therapy reduces healing time and symptom severity. Continue for 5-10 days.
- **Recurrent episodes:** Most effective if started within 48 hours of symptom onset (vesicle or prodrome). Shorter course than first episode.
- **Suppressive therapy:** If 6 or more recurrences per year, consider long-term daily antivirals (aciclovir 400mg BD or valaciclovir 500mg daily).
- **Supportive care:** Analgesia, topical treatments (Vaseline), keep lesions clean and dry.

Red Flags / Complications

- **Immunocompromised patients:** Higher risk of severe/disseminated disease. May need IV aciclovir. Refer to specialist.
- **Pregnancy near term (>34 weeks):** Risk of neonatal herpes. Maternal antivirals reduce transmission. Caesarean section advised if lesions present at delivery.
- **Meningitis/encephalitis:** Rare. Headache, neck stiffness, altered consciousness. Emergency referral required.
- **Disseminated infection:** Rare. Systemic symptoms, hepatitis, multi-organ involvement. Specialist care.

Patient Group Direction

for the supply/administration of

Aciclovir 400mg tablets

for the treatment of

Genital Herpes Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed

	equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of first episode and recurrent genital herpes simplex infection. Suppressive therapy for frequent recurrences (6 or more per year).
Inclusion criteria	Confirmed or highly suspected genital herpes (HSV-1 or HSV-2 positive or clinical diagnosis) Aged 16 years and over Able to give informed consent No contraindications or cautionable conditions (see exclusions/cautions)
Exclusion criteria	Hypersensitivity to aciclovir or valaciclovir Significant renal impairment (eGFR <30 mL/min; caution if eGFR 30-60) Severe hepatic impairment Immunocompromised patients, any presentation (HIV, chemotherapy, transplant, biologic or high-dose steroid therapy): refer to specialist Suspected disseminated infection, meningitis or encephalitis, or inability to pass urine, in ANY patient: emergency referral (999 or A&E), not a PGD supply Pregnancy at any gestation, or breastfeeding: refer to the GP or GUM clinic. A first episode in pregnancy needs specialist management, and

	suppressive treatment from 36 weeks is a prescriber decision
Cautions	Renal impairment (eGFR 30-60 mL/min): dose adjustment may be required Elderly patients: increased risk of renal impairment; adequate hydration essential Hepatic impairment (mild-moderate): monitor closely Dehydration: ensure adequate fluid intake; risk of crystalline nephropathy if dehydrated Neurological disease: rare but possible (confusion, tremor, hallucinations) particularly with IV formulation or renal impairment
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Aciclovir 400mg tablets
Legal category	POM
Storage	Below 25°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	First episode: 400mg three times daily for 5-10 days (depending on severity and response) Recurrent episode: 800mg three times daily for 2 days (starting within 48 hours of symptom onset) Suppressive therapy (6 or more recurrences a year): 400mg twice daily. Under this PGD a maximum of 3 months' suppressive supply may be made, after which GP or GUM review is required before any further supply
Quantity to be administered and/or supplied	First episode: 15 tablets (400mg three times daily for 5 days); a further 15 tablets, to 10 days, only where new lesions are still forming at day 5. Recurrent episode: 12 tablets (800mg three times daily for 2 days). Suppressive therapy: 56 tablets per 28 days, maximum 3 supplies (168 tablets) under this PGD before review
Maximum or minimum treatment period	First episode: 5 days, extended to 10 days only where new lesions are still forming at day 5. Recurrent episode: 2 days. Suppressive therapy: maximum 3 months (three 28 day supplies) under this PGD, then GP

	or GUM review before any further supply.
Adverse effects	Common (>1%): nausea, headache, diarrhoea, abdominal pain, rash (including photosensitivity) Uncommon (0.1-1%): vomiting, fatigue, fever, arthralgia Rare (<0.1%): renal impairment (elevated creatinine, crystalline nephropathy if severe dehydration), neurological effects (confusion, tremor, hallucinations, psychosis) usually with IV therapy or in renal failure, thrombocytopenia, hepatitis Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form and route, and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with aciclovir or valaciclovir tablets.
Follow-up advice to be given to patient or carer	This is not a cure for herpes; the virus remains in the body and recurrences are possible. Use condoms consistently to reduce transmission to partners, even if

	no symptoms present. Avoid all sexual contact during prodromal symptoms and active lesions (vesicles or ulcers). Discuss disclosure to sexual partners; consider partner testing and treatment. Consider HPV vaccination if not already vaccinated (if cervical screening due, attend regularly). If pregnant or planning pregnancy, discuss genital herpes management with GP/obstetrician early. Complete the full treatment course even if symptoms improve early. Seek medical advice if symptoms do not improve within 10 days, or if you experience severe pain, fever, or signs of systemic infection. Report any adverse effects via Yellow Card (https://yellowcard.mhra.gov.uk). If recurrences become frequent (6+ per year) discuss suppressive therapy with healthcare provider.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Herpes simplex virus infections
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Valaciclovir 500mg tablets

for the treatment of

Genital Herpes Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Alternative antiviral for first episode and recurrent genital herpes simplex. Better bioavailability than aciclovir (less frequent dosing). Suppressive therapy for frequent recurrences (6 or more per year).
Inclusion criteria	Confirmed or highly suspected genital herpes (HSV-1 or HSV-2 positive or clinical diagnosis) Aged 16 years and over Able to give informed consent No contraindications or cautionable conditions (see exclusions/cautions) Preferred in patients requiring less frequent dosing (twice daily vs three times daily)
Exclusion criteria	Hypersensitivity to valaciclovir or aciclovir Significant renal impairment (eGFR <30 mL/min; caution if eGFR 30-60) Severe hepatic impairment Immunocompromised patients, any presentation (HIV, chemotherapy, transplant, biologic or high-dose steroid therapy): refer to specialist Suspected disseminated infection, meningitis or encephalitis, or inability to pass urine, in ANY patient: emergency referral (999 or A&E), not a PGD supply Pregnancy at any gestation, or breastfeeding: refer to the GP or GUM clinic. A first episode in pregnancy needs specialist management, and suppressive treatment from 36 weeks is a prescriber decision
Cautions	Renal impairment (eGFR 30-60 mL/min): dose adjustment required; seek specialist advice if eGFR <30 Elderly patients: increased risk of renal impairment; ensure adequate hydration Hepatic impairment (mild-moderate): monitor closely Thrombotic thrombocytopenic purpura (TTP) / haemolytic uremic syndrome (HUS): rare risk in immunocompromised patients Adequate hydration essential to reduce renal toxicity risk
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Valaciclovir 500mg tablets
Legal category	POM
Storage	Below 30°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	First episode: 500mg twice daily for 5-10 days Recurrent episode: 500mg twice daily for 5 days (starting within 48 hours of symptom onset) Suppressive therapy (6 or more recurrences a year): 500mg once daily. Under this PGD a maximum of 3 months' suppressive supply may be made, after which GP or GUM review is required before any further supply
Quantity to be administered and/or supplied	First episode: 10 tablets (500mg twice daily for 5 days); a further 10 tablets, to 10 days, only where new lesions are still forming at day 5. Recurrent episode: 10 tablets (500mg twice daily for 5 days). Suppressive therapy: 28 tablets per 28 days, maximum 3 supplies (84 tablets) under this PGD before review
Maximum or minimum treatment period	First episode: 5 days, extended to 10 days only where new lesions are still forming at day 5. Recurrent episode: 5 days. Suppressive therapy: maximum 3 months (three 28 day supplies) under this PGD, then GP or GUM review before any further supply.
Adverse effects	Common (>1%): headache, nausea, dizziness, abdominal pain, rash Uncommon (0.1-1%): vomiting, fatigue, arthralgia, tremor Rare (<0.1%): renal impairment (elevated creatinine, crystalline nephropathy), neurological effects (confusion, hallucinations, psychosis), thrombotic thrombocytopenic purpura (especially immunocompromised), hepatitis, thrombocytopenia Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given

	• name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form and route, and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with aciclovir or valaciclovir tablets.
Follow-up advice to be given to patient or carer	This is not a cure for herpes; the virus remains in the body and recurrences are possible. Use condoms consistently to reduce transmission to partners, even if no symptoms present. Avoid all sexual contact during prodromal symptoms and active lesions (vesicles or ulcers). Discuss disclosure to sexual partners; consider partner testing and treatment. Consider HPV vaccination if not already vaccinated (if cervical screening due, attend regularly). If pregnant or planning pregnancy, discuss genital herpes management with GP/obstetrician early. Complete the full treatment course even if symptoms improve early. Seek medical advice if symptoms do not improve within 10 days, or if you experience severe pain, fever, or signs of systemic infection. Report any adverse effects via Yellow Card (https://yellowcard.mhra.gov.uk). If recurrences become frequent (6+ per year) discuss suppressive therapy with healthcare provider.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 24: the valaciclovir recurrent episode course is fixed at 5 days, 10 tablets (500 mg twice daily), the upper end of the SmPC range, in the dose, quantity and treatment period rows; the previous 3 to 5 day range with no criterion for choosing is removed.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover corrected to the supply of aciclovir or valaciclovir tablets for genital herpes (it read administration and Genital Herpes Management).

Maximum or minimum treatment period rows in both arms now match the dose and quantity rows: first episode 5 days extended to 10 only where new lesions form at day 5, recurrence 2 days (aciclovir) or 3 to 5 days (valaciclovir), suppressive therapy capped at 3 months under this PGD.

Records row template text separated into date, dose, form, route and quantity, and the adverse reaction bullet no longer runs into 'supplied via PGD'.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Exclusions rewritten in both arms: immunocompromise excludes for any presentation; suspected meningitis, encephalitis, disseminated infection or urinary retention is an emergency referral for any patient; pregnancy at any gestation and breastfeeding refer, not only 'near term'.

A quantity is stated for every regimen (first episode, extension, recurrence, suppression) in both arms, and suppressive supply under this PGD is capped at 3 months before GP or GUM review. The previous version gave one quantity for the first-episode course only.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

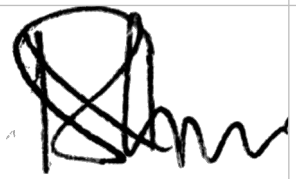
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The two arms previously carried different validity periods.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.